

## SECTION V

# Common Reactions to Relational Trauma

## CHAPTER 14

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# Vulnerability

### VULNERABILITY: A FOUR-LETTER WORD

At the core of most, if not all, relational wounding is a violation of our vulnerable selves—our young parts that represent the jewels of our personality. The vast array of protective responses that develop as a result of this breach are primarily designed to ward off the pain carried by our exiled parts. People often equate being vulnerable with being hurt and, therefore, try to avoid being vulnerable at all costs. I feel this is a *distorted belief* that developed as a result of trauma and that we, as therapists, can help our clients revisit this view from another perspective, one that corrects the distortion and includes the Self.

My clients frequently react to my suggestion that they should try being vulnerable with their partner, friends, or children as if I were speaking to them in a foreign language. They appear to have an allergic reaction to the idea, uttering incredulous responses such as: “Are you kidding me?” “Why would I do that?” “It’s just a sign of weakness,” and “That’s the reason I got hurt in the first place—don’t you get that?”

The definition of *vulnerability* in the Oxford English Dictionary is: “The quality or state of being exposed to the possibility of being attacked or harmed, either physically or emotionally.” Logically, vulnerability would be experienced as something to be avoided when it’s associated with being hurt or wounded instead of being associated with the expression of what we feel deep inside ourselves.

Brené Brown speaks to our cultural perception of vulnerability as predominantly a weakness and redefines it as a strength instead of a deficit. She says: “Vulnerability is the core of all emotions and feelings. To feel is to be vulnerable. Vulnerability is the birthplace of love, belonging, joy, courage, empathy, and creativity” (Brown, 2012, pp. 33–34). She further defines vulnerability as uncertainty, risk, and emotional exposure.

I agree that vulnerability is a superpower, not a weakness. *I see vulnerability as comprising two different components: (1) Self-connection, or the ability to connect with what we feel or to our truth; and (2) the ability to share it with another person.* To me, being vulnerable is being genuine or authentic with ourselves as well as with another. It means not wavering or leaving our truth and not worrying about the other person and their reaction or response to the expression of our true feelings.

A more common response from a trauma survivor might be: “If I’m honest and vulnerable, I’ll lose this relationship. Do I disconnect from what I know to be true to save this relationship? Have I ever really known or been able to connect with what I feel?”

Being vulnerable can feel like a big risk, as Brené Brown describes, especially when you have repeatedly been slammed, shamed, criticized, or attacked for expressing yourself in the presence of another person (Brown, 2012). This, unfortunately, is all too common for complex trauma survivors, many of whom have found that connecting to and sharing what they feel with another person will only lead to something horrible; hence, they don’t do it. Once you have been repeatedly hurt in this way, you develop an elaborate protective mechanism to keep the emotional pain away. *One of the great gifts of IFS therapy, as it relates to complex trauma, is its reversal of the suffering that resulted from our truth being attacked by another.*

To be seen and known, warts and all, and to be who you truly are—openly and freely in connection with another person—can certainly be scary. But it also can be exhilarating, amazing, and enormously freeing when it goes well. In its most basic form, to be vulnerable is to be intimate with our self and others.

I spent many hours in therapy before I could fully embrace this idea of loving who I am and feeling confident and comfortable to freely express this to the people in my life—especially as a gay man with a trauma history. Yet the rewards have been immeasurable, as evidenced by a higher happiness quotient. The older I get, the more “myself” I become and the more I’m able to express this truth to others with clarity and confidence. My journey is by no means over; instead, I will always be in therapy and continue to evolve, expand, and unfold in wonderful and surprising ways.

## REACTIONS TO FEELINGS

Many trauma survivors who still carry portions of their trauma inside themselves don’t fully understand yet that it’s their perpetrator’s reaction to their feelings that’s the real problem, not the expression of their vulnerability. Remember: Complex PTSD, as we’ve defined, is typically regarded as a violation of a relationship. I believe that the problem developed because of the way the other person responded, how they reacted, and what they said or did. “It’s about their parts, not yours,” I tell my clients. A burden is created because a part got hurt and was forced to hold the pain around *the response* to the expression of their truth. Once again, it’s not the expression of our feelings that’s the problem; *it’s the reaction to them*. And this gets confused all the time, particularly with children, who typically will take responsibility for what was said and done to them, especially when the other person—usually an adult—doesn’t.

It boils down to confusion around the issue of *responsibility*. Who's responsible for what? During therapy sessions, I sometimes hear clients' parts say, "I'm sick of you! How many times do I have to tell you to shut up already?" (much like a parent yelling at their child) or "I'm an idiot. It's my fault. I'm such a loser" (much like a child will respond to verbal abuse). These types of responses are often critical internalized perpetrator parts trying to take control of something that feels scary and painful. When kids are left alone with their traumatic experiences, they need to make sense of the situation, and they often take responsibility for what was done to them. *This is how distorted beliefs get embedded and how vulnerability gets blamed as the problem.*

## VULNERABILITY AND CONNECTION

Gabor Maté talks about two basic needs that all human beings have: (1) the need for attachment, and (2) the need for authenticity (knowing what we feel and being able to express who we are; Maté 2011). He further describes what happens when the two are not aligned. When children receive information about themselves that doesn't align with their authentic Self, they have to choose between the two; they will often choose attachment over authenticity for the sake of survival (Maté 2011). They disconnect or suppress their true Self to survive. Maté believes that repeated misalignment or disconnection can later manifest itself in mental health problems we face as a society, such as addictions and physical illnesses. This is a bold statement to make—that addiction and physical illness are rooted in relational trauma—yet one I totally agree with.

In complex PTSD, clients will repeatedly receive messages that are not aligned with their vulnerable, authentic truth. They disconnect from their Self and internalize the messages they receive. The feelings are often held in their exiled parts, and an elaborate system of protection gets created to protect from the misalignment of their truth and the isolation from their Self. When trauma survivors do receive a message that is aligned with their truth or Self, it's often difficult for them to take it in, as they still hold the belief of a prior assault. Being told "I think you're kind and loving" doesn't ring true to them, and it's hard to receive when they have parts that carry beliefs like "You're worthless," "You're ugly," and "You're unlovable."

One of the main goals in IFS therapy is to help clients reconnect so they can trust their feelings again and regain faith in the Self. For example, when a client says something about me or notices that I'm distracted or tired, I try my best to validate their experience. I'll respond: "Yes, you're correct—my mind did wander just then. Let me take a moment to see if I can get a sense about what's going on for me" or "Yes, you're right. I did react when you told me how I was feeling." Whenever clients are able to speak their truth and we are able to validate their experience, we help them reconnect with

their gut feeling, allowing them to trust their inner wisdom again. This trust in Self-leadership, after all, is what was destroyed as a result of the trauma.

Were you accepted for who you are? Were the messages you received aligned with your gut instinct, or did the reaction to your vulnerability present as shaming, critical, or attacking? Relational trauma is about the pain we carry from the perpetrator's reaction to who "they" thought we are, what they thought we should feel, or who they thought we should be. It's the way we're taken advantage of, overpowered, rejected, and alienated that causes the wound. Relational trauma is a violation of healthy loving connection, true intimacy, and the acceptance of our true vulnerable selves.

### **VULNERABILITY FROM PARTS OR THE SELF?**

It's important to differentiate where vulnerability is coming from. Does it originate from the Self or a part? When younger parts express themselves through feelings, beliefs, or physical sensations, there's often an intensity and agenda attached to it. They're desperate to be seen and heard, and they typically seek redemption from a prior hurt. Unfortunately, when parts express vulnerability in this way, they're likely to activate a part in the other person and reexperience or relive their original hurt. Remember: Parts beget parts. When we respond from a part, we're likely to get a part response in the other. Exiled parts typically don't get the response they're looking for; consequently, the wounds are commonly reinforced. You might very well hear a client's vulnerable wounded part say: "I'm so lonely and afraid. Please stay with me, and promise you'll never leave me."

On the other hand, when vulnerability is expressed from the Self, it is much more likely to get the desired outcome. The Self has an infinitely higher likelihood of getting a corrective or reparative response. Wayne Baker, a dear friend and IFS colleague, says the "Self is the ultimate protector of our vulnerability" (personal communication, September 16, 2019). When we are in Self-energy, our guard drops, our protectors relax, and we can connect to and speak for our feelings and deepest thoughts from a place of strength and power. Here, we are being vulnerable from a place of strength, not desperation. The Self also has better judgment about whom we should share our vulnerability with—whom we can honestly trust versus whom we think can save us.

IFS helps our clients heal from the anguish their parts have endured. We can show our clients the goodness in themselves again and enable them to let go of their perpetrator's harsh words, actions, and beliefs carried by their parts. Trauma makes intimacy and vulnerability seem like a bad idea or scary notion, and the Self helps wounded parts see that they're not the problem after all and begin to experience vulnerability as a strength and a joy, not a weakness or a poor choice. *Trauma blocks self-love, and Self-led vulnerability fosters greater connections and deeper intimacy.*



## CHAPTER 15

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# Reactions to Trauma

### COMMON RESPONSES TO RELATIONAL VIOLATIONS

Relational trauma is a violation of our vulnerability, truth in feelings, and trust in the other. A resultant protective cascade evolves in an attempt to keep the sorrow at bay. Here's where therapists spend most of their time: trying to get stubborn or resistant protective parts to relax and allow access to the wounds they're guarding.

Here is a list, not exhaustive by any means, of protective parts that therapists frequently encounter when helping complex PTSD survivors recover:

- **Dissociative parts**
- **Suicidal parts**
- **Numbing parts**
- **Shaming parts**
- **Addictive parts**
- **Passive parts**
- **Perpetrator parts**
- **Substance-using parts**
- **Sexually excessive parts**
- **Sexually avoidant parts**
- **Depressed parts**
- **Panicked parts**
- **Anxious parts**
- **Critical parts**
- **Vigilant parts**
- **Storytelling parts**
- **Worrying parts**

- **Controlling parts**
- **Shy parts**
- **Rigid parts**
- **Rageful or angry parts**
- **Intellectual parts**
- **Obsessive parts**
- **Conflict-avoidant parts**

Next, we'll examine some of the more common protective responses and explore the unique ways these parts interact with each other while protecting their vulnerable counterparts.

### **DISSOCIATIVE PARTS**

As previously discussed in [section 3](#), parts that disconnect, numb out, or remove themselves take on these roles when danger is perceived. Information in the amygdala is redirected from the lateral to the basal nucleus, which activates the ventral striatum; the PFC becomes overinhibited; and the dorsal parasympathetic nervous system is activated.

I believe that dissociation happens on a spectrum. All of us daydream, occasionally space out, or are overly preoccupied at times, which is at one end of the dissociative spectrum. On the other end of that spectrum are those who constantly leave, lose chunks of time throughout the day, or even split off into discrete identities that function independently and lack awareness of one other. As Dick Schwartz often says, IFS sees dissociation as a part, not as a pathological process.

When I notice dissociation in a client, I might remark: “Emily, I noticed that a part of you just checked out. Are you aware of that? Let’s learn more about why that part felt the need to do that.” Remember that most dissociative, avoidant, or out-of-here parts aren’t particularly interested in engaging in a conversation. Their agenda is to protect by disconnecting, not to converse. The way IFS gets around this is not via grounding techniques, like some other therapies suggest (which is essentially shifting out of one part into another one), but by talking directly to the part and hearing its position. I find this technique more efficient, effective, and respectful to the client’s parts.

For example, I might say: “So you’re the one who took Emily out a moment ago, is that right?” Waiting for a response, I will often continue the conversation, knowing that it takes dissociative parts a few moments to engage. My next words may be: “I trust that you had a good reason to jump in just then. I suspect that you’ve needed to do this for quite some time. I’m truly interested in hearing more



about that.” I’ll likely see the client’s body gradually shift and slightly soften. This suggests that we’ve made a connection and can begin a dialogue.

Parts jump in because they feel they have to. Engaging them in conversation allows them to be heard, known, and validated. Grounding strategies, from the IFS perspective, essentially call on another part within the system to show up and override the original part. Here, dissociative parts can feel disregarded or unappreciated. Keep in mind that leaving is often the best solution in a frightening or overwhelming traumatic moment. Instead of getting frustrated and annoyed by these parts, I’ve grown to love them for their tenacity and persistence.

## SUICIDAL PARTS

The extreme example of a part abandoning an untenable situation is suicide. Even though we know in IFS that every part has a positive intention, our profession and culture are programmed to stop it at all costs. I’m aware of the increased suicide rate within the adolescent population at present, which is likely due to the effects of technology and the lack of love, connection, and guidance experienced by this age group. Of course, I’m not in favor of suicide, but I do believe we are approaching it in the wrong way culturally. Sure, we need to differentiate whether or not the suicidal thoughts are connected to a biologically based depression and treat it accordingly. But more importantly, I believe that we need to understand the intentions of these desperate, last-resort parts.

When you’re truly able to engage suicidal parts with open-hearted curiosity, you will quickly learn that dying is often the last thing on their minds. *They tend to show up when the pain is high and the access to love is minimal or nonexistent.* Often, these parts are young and don’t believe that suicide will lead to their removal. They’re the ultimate escape artists. “If it gets bad enough, I have what it takes to stop the pain,” is what we hear over and over again from these heroic parts. Appreciate them for the tremendous burden of responsibility they carry for the system.

For therapists to be able to hold this compassionate position, they need to work with their own parts that get activated by these wishes and free themselves from the professional burdens that bind them. It’s normal to think things like “It’s my job to prevent Hilary from killing herself. If I don’t, I’ve failed, and I will most certainly get sued.” When physicians witness clients dying, they usually don’t blame themselves when this happens; it’s considered a normal aspect of life. Therapists, on the other hand, take on their profession’s view and often assume more self-responsibility than is warranted in these situations.

We also need to clear any personal legacy burdens we might carry. For example, my niece accidentally committed suicide a few years back. So every time one of my clients’ suicidal parts

shows up, I need to keep that part at bay—the one that tries to jump in and convince my client of the potential devastation it would cause for the people in her life. Instead, we all need to take these parts seriously. When we are more sensitive to the significance of their job and the tremendous pain these parts are trying to push away, things tend to go much better; usually, these suicidal parts don't escalate but soften because they finally feel listened to.

### **THE PERPETRATOR AND PASSIVE PARTS: INTERNAL PROTECTIVE POLARITIES**

When a vulnerability is violated and the emotional pain associated with the experience is predominately held in exiled parts, two common protective responses develop independently and in polarization with each other. These are passive parts and perpetrator parts.

Passive parts can be protective and dominate one's life, even though they present as meek, quiet, under the radar, or even hidden. They often run the show from the sidelines. Passive parts can originate directly from exile energy. You may hear them say: "I'm too run-down or beat-up to respond to anyone or anything. I'm helpless." Or they can be protective in nature, remarking: "If I stay quiet, I'm less likely to draw attention to myself and get hurt. Doing nothing is safer than standing out and speaking up. If I stay small, my father won't yell at me."

These parts are physiologically rooted in parasympathetic blunting or withdrawal and tend to activate therapists' parts. Yes, I speak from personal experience here. I can often feel frustrated with a client's seeming lack of motivation to change, their inactivity or inability to mobilize. I may ask questions such as: "Were you able to tell your husband that you'd like to be involved in making decisions about where the family goes on vacation? Were you able to tell your daughter not to talk to you with an abusive tone? Did you inform your boss that you can't work another overnight shift?" No, is the common response I'll hear. My parts get frustrated that the client isn't speaking up, standing up, or even showing up like they probably should, due to the dominance by their passive parts that are working overtime to keep exiled parts out of harm's way. I've had to address the parts of me that get activated by passivity. I now understand that passivity in my history meant that I'd put myself in harm's way. My parts felt the need to act and respond with lightning speed to keep me safe. Being passive was terrifying for them—akin to being a sitting duck in a shooting gallery.

To better illustrate this point, consider the following exchange between me and a client:

"I'm running out of places to hide," Will said. "I've never really been true to myself, and it constantly blows up in my face. My needs have never mattered, and my regrets are piling up. I only have about 20 years of life left, and I can't keep running like this."

“I think it’s great that you’re wanting to make some changes in your life, Will. But let’s not throw these parts under the bus just yet. Let’s ask them more about their story,” I said.

“They seem to be connected to my drinking part and the way it avoids feelings at all costs too. They seem pretty young,” Will said. “I keep hearing ‘We can’t speak up,’ ‘It’s not allowed,’ ‘If we talk, then we’ll get hit and yelled at even more,’ and ‘It’s not an option for us.’”

“Let them know that we’re hearing what they’re saying, Will. It sounds like speaking up was scary for them, and if they did, they clearly would get hurt,” I responded.

“This is making so much sense to me,” noted Will. “They learned from a very young age to ignore feelings and never speak up about anything.”

“They had to, Will. It sounds like it was a matter of survival for them,” I said.

“It totally was,” Will said, crying quietly.

Some exiled parts learn from a very young age that keeping quiet is a better option than speaking up. This message of passivity is internally imprinted and carries over into adulthood, often getting played out in a myriad of ways. The result is that many complex PTSD survivors remain passive, present as a victim, and are unable or unwilling to speak for their needs. To wit, consider my continued conversation with Will:

“Will, it’s really important that we’re learning more about why it’s been so hard for you to be true to yourself and to have your needs matter in your adult life. Let these parts know that you’re beginning to see why they were so terrified to speak up and that we understand why they needed to keep running away from feelings to stay safe,” I said.

“They like that I’m connecting to this. They’re saying: ‘It’s about time,’” Will replied.

“I believe that our feelings are our truth barometers,” I said. “They keep us connected to our moral compass and true self. You couldn’t do this as a child if you were terrified all the time. You can only know what’s true inside when it feels safe enough to do so, and you can only speak up about your emotions and let others have their feelings when the people in charge create the space for that to safely happen.”

“I’ve never even thought about that before,” commented Will. “It never seemed possible for me to have my feelings and let them have theirs; that would have been a disaster.”

“I believe that was true back then, Will. But if you and I spend time with those younger parts—the ones that needed to disconnect and stay silent to survive—we can help them let go of what they’re holding. And you’ll be much more capable of doing that now in your life.”

“That’s hard to believe. But count me in,” Will said. “As I said before, I’m running out of time.”

Perpetrator parts are also quite prevalent in relational trauma and represent the other side of the polarization. Most trauma survivors incorporate both passive and perpetrator parts, and both tend to be hard to acknowledge and embrace. Perpetrator parts are the parts of the client that take in the energy and effectiveness of the perpetrator. These parts are usually sympathetically activated and hyperaroused. Our clients see and, unfortunately, feel how effective the perpetrator's dominance and power was because they were the recipient of that energy. Young protective parts tend to take on this energy in an attempt to feel big and powerful. Their rationale goes something like this: "Wow, if I attack people as my uncle attacked me, no one will mess with me anymore."

When dealing with perpetrator parts, it's important to differentiate what percentage is the client's part (i.e., that which was internalized) and what percentage is associated with a legacy burden (i.e., energy that belongs to the perpetrator and is attached to the client's system; for more on legacy burdens, see [chapter 25](#)). Usually, it's a combination of both the client's part and the perpetrator's energy affixed to the client. The good news here is that both can be released.

An important question to help you differentiate between the two is: "Does this energy inside of you belong to your uncle, or is it yours?" You might hear a response along the lines of: "Forty percent is mine, and the rest belongs to my uncle." A useful response would be: "If this order makes sense to you, let's release the portion that belongs to him first, and then let's help the portion that belongs to you."

Connecting with perpetrator parts can be one of the most painful experiences for trauma survivors. I regularly hear phrases like "I can't believe I did the very thing that was done to me," and I've felt this personally as well when I've yelled at my children as a result of a triggered perpetrator part of me. My internal critic then jumps in and gets extremely harsh toward this angry perpetrator part, admonishing it with statements such as "Look at what you just did. You know better than this. You're a therapist, and you teach appropriate behavior for a living, you jerk."

*As difficult as it is sometimes, these perpetrator parts also need to be loved and appreciated for their good intentions, not for their effect.* I've learned to appreciate the way my perpetrator part jumps in to protect the young one inside me that gets terrified when my two children fight with each other. I have also become grateful for my internal critic, who works hard to protect my children from my perpetrator's rage.

## CRITICAL PARTS

Internal and external critics are commonplace in trauma and are often internalized perpetrator energy that are rooted in some form of judgment or verbal abuse. They excel at hurling insults: "You're

going to amount to nothing. You're so damned lazy. You make me sick." Unfortunately, the recipient of these harsh words knows the impact and harm they cause, which can often be felt like an attack on the soul.

However, it's important to help your clients access the intention behind these critical parts. They can be preemptive (a manager) or reactive (a firefighter) in nature and are trying their best to protect your client by getting them to behave, be perfect, not stumble again, or push potential assailants away.

Asking them where they learned to do this job—and if they're interested in discovering a new way of helping—is a relational approach to working with extreme critics that I learned from Chris Burris, a senior lead trainer at the IFS Institute. They typically have a bad reputation in society, in families, and with therapists, which they are well aware of, and when we draw attention to their activity, they inevitably feel shame, which fuels their fire. Again, kindness toward the critic's positive intention goes a long way toward gaining access to the real wound—the one inflicted upon by the original perpetrator, not the well-intended pupil.

Emotions inform our truth and help us stay aligned with what's right for us, which is a textbook description of self-connection. We have no regrets when we connect to what we feel and live and speak our truth. However, in trauma, polarities often form between passive parts and critical parts or perpetrator parts that help push away the pain connected to our overwhelming experiences. One of my clients confessed: "I'm constantly stuck between fighting too hard and never giving up or stepping back and letting everyone else decide for me. I never seem to find the right balance." I assured him that IFS helps resolve these polarizations by joining these sides, appreciating these parts' efforts, and offering to release the anguish they're protecting.

## CHAPTER 16

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# Substance Use: Searching Outside for Protection

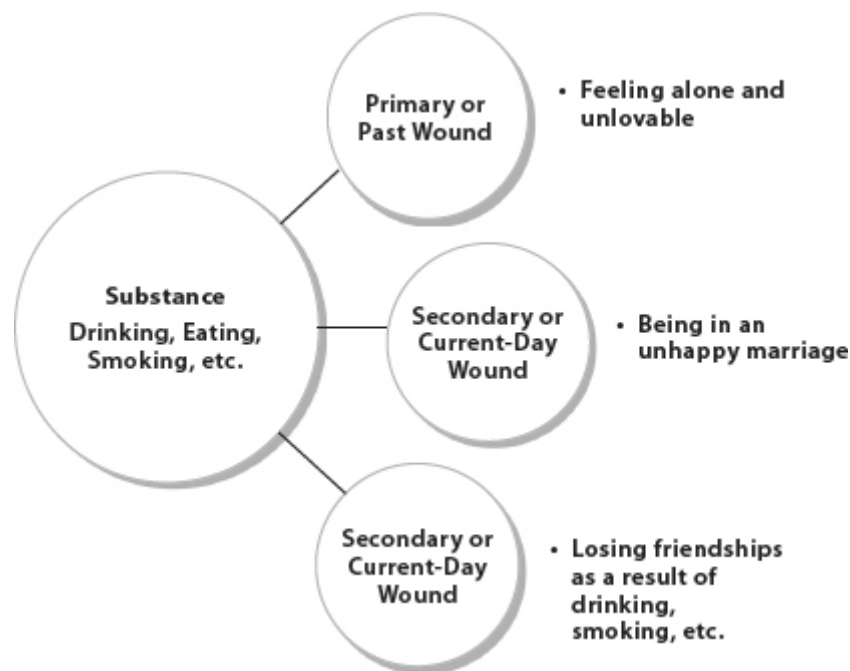
It's rare to work with relational trauma and not encounter parts that are connected in some way to an addictive cycle. Many internally damaged systems will look externally for relief, escape, and protection, especially when the internal system has not experienced safety and love early on in life. This is often the case when parts seek solutions to their problems with potentially addictive substances and behaviors, such as food, drugs, sex, technology, and gambling, to name a few. When it comes to these parts, we want to follow the client's internal wisdom about the order of healing. These systems tend to be complex and often have multiple dimensions to them, including facets connected to the past as well as the present. For example, *it's fairly common for there to be more than one substance-using part, and they tend to protect more than one wound.*

Remember that our goal in IFS is not to get the part to stop using their substance of choice; rather, it's to learn about their role, fear, or positive intention and to offer an alternative solution to their problem. This goal nicely aligns with a statement made by Johann Hari in his 2015 TED Talk on addiction: "The opposite of addiction is not sobriety. The opposite of addiction is connection." Be mindful that the overall solution usually involves healing an original wound, commonly rooted in the past, as well as addressing the secondary problems that developed, typically related to circumstances in the present day (as shown in the graphic on the next page).

For example, you may hear a client say: "I'm afraid that if I stop eating, I won't be able to suppress my sexual urges and I'll have another affair," which addresses a problem in the current day. You may also hear the client acknowledge the underlying wound that most likely originated in the past: "I'm afraid that if I stop eating, I'll feel unloved, unimportant, and worthless all over again, just like I did as a kid." Substance-using protective parts often end up protecting an original wound as well as several secondary wounds, some of which develop as a result of the substance itself.

Not only is it common for a substance to protect multiple wounds, but it's also customary for several different parts to be involved in the protective cycle. For example, with sexual addiction, there's often an activated or hyperaroused part that is driven to conquer, be sexual, and achieve the release of orgasm. There is also commonly a restrictive or hypoaroused part that wards off the

residual feelings of guilt after a conquest, as well as a part that desperately wants to have a normal sexual connection with someone.



To demonstrate this phenomenon, consider the case of a client named Jon. He had been seeing me for a while, primarily to address his issues with eating that seemed to be related to a contentious divorce with his wife and a history of being bullied at school as a child. Jon would frequently inquire about entering an inpatient program to help him lose weight, something he had struggled with for most of his life. As we began to explore the positive intention of his eating, he quickly discovered that he had many different parts that were organized around food. He identified a part that would “eat to soothe,” remembering how he would consume an entire bag of candy bars after being bullied, even though it upset his stomach. Jon also described a “warrior” who would eat to gain strength and have enough fuel to get through the next day at school after an attack. He further described a “last supper” part who ate as much as possible on Sunday night in anticipation of starting a diet the next morning in the hopes that his wife would find him attractive again and stop berating him. Last, Jon named the “mayor,” a part who knew how to “take care of everyone in town” and knew what was needed to eat right and live a healthy lifestyle.

When multiple addictive parts get uncovered, don’t automatically assume they’re all protecting the same wound. Sometimes one part protects more than one wound inside, while at other times, different parts protect various exiles, as was the case with Jon.

## PARTS, HABITS, AND BIOLOGY

With addictive substances, we help clients identify and heal the underlying wound associated with the protective behavior. But I have come to learn that it's also important to help them address the *habit* and *biological consequences* connected to the substance they're using. We may hear a client say: "I always have a cigarette after I eat and when I drive in the car" (i.e., a habitual behavior) or "How do I safely stop drinking so that I don't go through withdrawal or have a seizure?" (i.e., the biological effect).

I also like to think about the difference between addiction and dependence as they relate to parts, habits, and biology. *Addiction* is defined as compulsive substance use despite potentially harmful consequences and is often accompanied by an inability to stop using it. *Dependence*, on the other hand, occurs when the body adapts to a substance, requiring more of it to achieve the desired affect (i.e., tolerance) and eliciting specific physical or mental symptoms if the drug is abruptly stopped (i.e., withdrawal; National Institute on Drug Abuse, 2020). *I believe protective parts and habits are more often associated with addiction, whereas biology is connected to dependence.*

In most cases, we focus on healing the wound first and secondarily look at changing the habit and addressing biological withdrawal. When I first started working with addictions using IFS, I assumed that healing the wound would spontaneously take care of the habit, as well as the biology. But unfortunately, this was not necessarily the case. Sometimes habits turn out to be protective parts that haven't let go of their job because they weren't informed about an unburdening that took place. Habits can also be entrenched, taking a long time to change, and they may or may not be associated with a protective part.

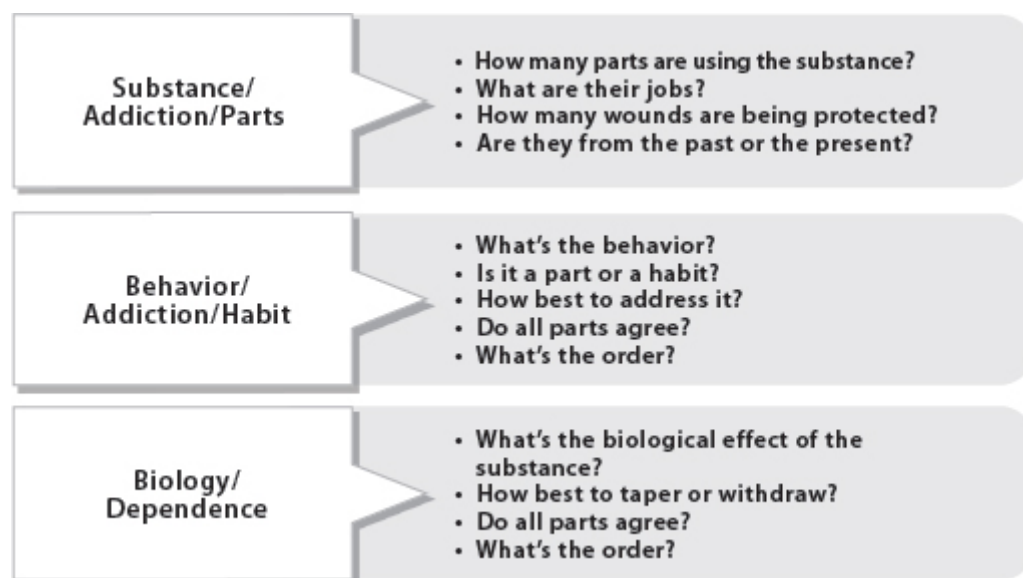
Having your client ask usually helps to differentiate between a habit and a part. "Jazmin, take a moment and ask inside if any part knows anything about or is connected to your eating dessert after dinner." Jazmin responded: "The part says it usually starts out eating to get away from stressful or overwhelming feelings, but at night, the habit kicks in and just keeps eating." When parts are uncertain or don't take responsibility for the behavior, it's likely a habit. In addition, withdrawing from some addictive substances (i.e., addressing biology) may require the support of a medical specialist to taper safely and to help prevent distressing or dangerous side effects.

Again, it's important to address all of the facets associated with the substance in question. Is food restriction connected to a protective part that's trying to prevent an exile from feeling unloved? Is it associated with a habit? (Statements like "Ever since college, I stopped eating breakfast and lunch" or "I was always so busy; it just became part of my normal routine" can confirm this.) Or is it a symptom of decreased appetite that is related to a biological depression?



Trust the client's internal wisdom regarding the order they assign to addressing the habit and biology. One of my clients was clear that he needed to decrease his alcohol intake before he was able to honestly address feelings connected to his retirement. Another client had a part of her that didn't trust her Self enough yet—a part that wanted her to “walk the talk.” It requested that she first start exercising and enter Weight Watchers before it would trust her enough to access the feelings of inadequacy. Yet another client of mine was able to unburden the little boy inside of him who was raped by his uncle as a child. As his exile started feeling the love and kindness it was robbed of by his uncle, his system decided to enter an outpatient eating-disorder program to address some of the residually embedded behaviors that were still present in his life.

In IFS, as it relates to the addictive process and trauma, we explore all the different parts that are connected to the substance at hand. We also examine the various wounds underneath the parts. Further, it's important to address the habits, rituals, routines, addictions, or cravings that have been established around each particular substance and to help the client develop a plan for addressing them. We also consider dependence or the biological realities of each substance and help our clients safely decrease using the substance when the timing is right for the system.



To demonstrate IFS's efficacy here, consider the case of my client Rebecca. After working with Rebecca's pot-smoking part for several sessions in a row, she was able to acknowledge the consequences of the part's behavior:

“I know that it's dangerous to drive our boat when I'm stoned. My wife gets on my case, especially when the kids are on board,” Rebecca told me. “My pot-smoking part really likes to be seen by me; it feels my love and support. She's willing to rein it in. She says my love gives her the strength to try and cut down.”

Rebecca then got misty-eyed and noted: “Wow, I just realized the connection—that I’ve been smoking for years because I never felt loved by my parents growing up. They were so caught up in their own stuff that I never felt their love.”

At this point, I offered the invitation: “Rebecca, let the pot-smoking part know that we can help heal the younger part that didn’t feel loved.”

“Oh, my goodness, it’s like I’m getting this for the first time,” Rebecca replied. “When I love the pot-smoking part, she feels that love and her heart opens up, which allows me in turn to love my wife and kids even more. She wants to stop smoking because she also loves them and doesn’t want to do anything that would put them in harm’s way.”

Rebecca made an important connection, followed by asking these crucial questions: “Healing involves connecting to the goodness in all of us. It involves love, even for our parts. The pot-smoking part can block love, needs love, and can even offer love, right?”

“Absolutely,” I responded, as I started to get teary-eyed.

### **ASSOCIATED PARTS: CRITICS, CARETAKERS, AND THINKERS**

When addressing parts that use substances in the service of protection, there’s often a host of related parts that get activated in connection to the wound. These are critical or shaming parts, caretaking parts, and parts that intellectualize or tell the story. This grouping of parts makes it much more challenging to receive permission to access the wound. For example, addictive parts will feel bad about their behavior and the trouble they’ve caused, both internally (to the system) as well as externally (to the people around them). It’s common for internal critics to show up and shame the addictive part for its behavior too.

Some parts caretake and desperately try to please, fix the problem, and offer guidance in the service of keeping the pain away. There are also those unrelenting parts that hold the story and remain in place, attempting to think and figure things out. These parts are notorious for having a hard time unblending and providing space, but persistence, patience, and compassion usually pay off in the end.

## CHAPTER 17

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# Shame Cycles Associated with Trauma

With complex relational trauma, shame is ubiquitous and becomes a complicated system unto itself. When we're vulnerable, connected to our truth in feeling, or connected to our power and someone reacts harshly or responds to us in an abusive way, they sever our Self-connection. Additionally, they activate or help create parts in us that hold or carry feelings of worthlessness, self-loathing, or being "less than." Thoughts like "I'm bad," "I'm wrong," or "I must be the problem" emerge.

Remember: *Shame develops when vulnerability is relationally violated.* In other words, shame is the anti-Self or the opposite of love and connection. Once during a session, an intuitive client told me: "When shame dominates, trauma wins. It gets in the way of me being me. It blocks me from being able to do what I'm capable of. When I believe the bad, I block the love I have for myself and the others in my life." Truer words were never spoken, proving that we can often learn more from our clients than they can from us.

There are two shame cycles I commonly, though not exclusively, come across as a result of repeated relational betrayal. One originates from parental or authority figures who frequently engage with children from a critical or verbally abusive part. The other comes from caregivers who respond from a part of themselves who is neglectful or abandoning of others. The critical parts in caregivers are typically harsh, hyperaroused, or sympathetically activated, while the neglectful, abandoning parts are more often parasympathetically blunted. Both represent triggered or blended states within the caregiver.

### CRITICAL SHAME CYCLES

Growing up in a critical, strict, verbally abusive environment will often produce an exile that holds shame and feels unworthy, bad, or unlovable as a result of the damaging words it has absorbed from a parent or bully. The negative feelings that result are predictable, with clients telling themselves "I'm rotten" or "I'm worthless." Being verbally abused not only produces an exile who carries shame, but it also produces a verbally abusive *internal critic*. Similar to the origin of perpetrator parts ([chapter 15](#)), this critical part typically originates in one of two ways: (1) it can become a part of the client that takes on the energy of the critical person in their life, or (2) it can represent the external person's

energy that affixes to the client's system but is not necessarily a part of the client per se. Again, we refer to the latter as a legacy burden.

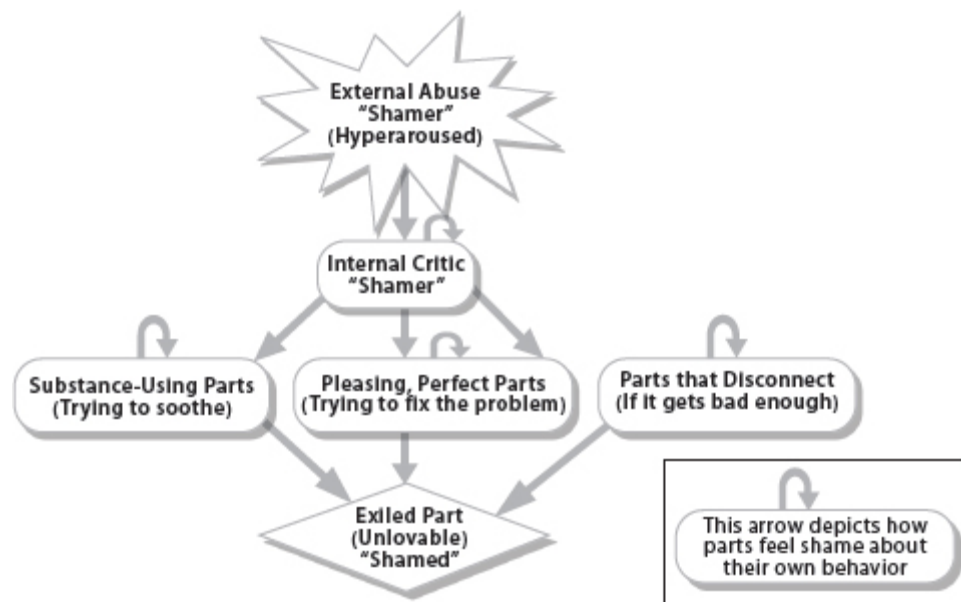
When the internal critic is a part of the client, it adopts a protective function. It often becomes critical of other parts within the client's system in an attempt to get them to "shape up and stop their bad behavior" in the hopes that the shamed exile won't get activated again. Common critical thoughts are "Your eating is the reason people hate us. If you stop, they won't make fun of us anymore" and "What's wrong with you—it's so obvious, don't you get that?"

Other protective parts also commonly show up as a result of being in a verbally abusive environment. There is often a *substance-using part* that tries to soothe the pain, a *pleasing or trying-to-be-perfect part* that attempts to fix the problem by getting people to like them, and a *disconnecting part* that says: "I'm out of here. I'll leave (dissociate) or even kill us all if things get bad enough."

To complicate things even further, each of these protective parts within the system self-perpetuates the shame cycle because they *feel bad about their own behavior*. They foster shaming thoughts, such as "I know my drinking keeps causing a great deal of pain to my family," "I keep trying to get people to like me, but I'm constantly failing," or "I know that leaving all the time pushes away the people I love the most." Here, shame begets more shame. These protective parts, which try to prevent the shamed wound from being triggered, are often ashamed of their behavior. The internal critic part often feels the worst of all, as it knows better than most how painful it is to be the recipient of criticism.

In my experience, there's nothing worse than being aware that you're perpetrating in a similar way that you've been perpetrated upon. Some of the worst moments in my life involve raising an angry voice to someone, just as I was yelled at in my childhood.

Note that, as the therapist, pointing out the effect of this vicious cycle can inject more shame into the client's system. It is our job to compassionately create a space of gentle exploration without judgment, to help these parts unload the shame they carry toward themselves and the other parts within the system they have shamed. Here is a diagram of a critical shame cycle produced by an external verbally abusive critic.



As you can see, this is a very intricate and convoluted cycle that perpetuates and reinforces shame produced by a critical outsider. In her best-selling book *Daring Greatly*, Brené Brown states that “shame is the fear of disconnection” and that “we are psychologically, emotionally, cognitively, and spiritually hard-wired for connection, love, and belonging” (2012, p. 68). She further adds that “shame is the intensely painful feeling or experience of believing that we are flawed and therefore unworthy of love and belonging” (2012, p. 69). I believe her definitions describe many of the exiles we work with in IFS, especially when we treat clients who have repeatedly been relationally criticized. The key takeaway here? *Shame is one of the primary wounds created when our vulnerability is attacked.*

### NEGLECT SHAME CYCLES

When children repeatedly encounter parental neglect, deprivation, or abandonment, they can feel ashamed and alone. This results from a lack of interest, love, and connection from the people they hold dear and rely on the most. It’s easy for kids to personalize this disinterest and feel unlovable, unworthy, and awful about themselves.

I like how Ken Benau describes this type of shame: “To seek special care and love when we are hurting and to have those needs met is to know, without words, that we are both loved and ‘good.’ To repeatedly not have our desire for care met, particularly when we are most vulnerable, is deeply shaming.... *Care sought and unmet is shaming*” (Benau, 2019).

Bessel van der Kolk states that human contact and attunement are the wellsprings of physiological self-regulation, and the promise of closeness often evokes fear of getting hurt, betrayed, and abandoned when shame is involved (van der Kolk, 2014). He further explains that unresolved trauma

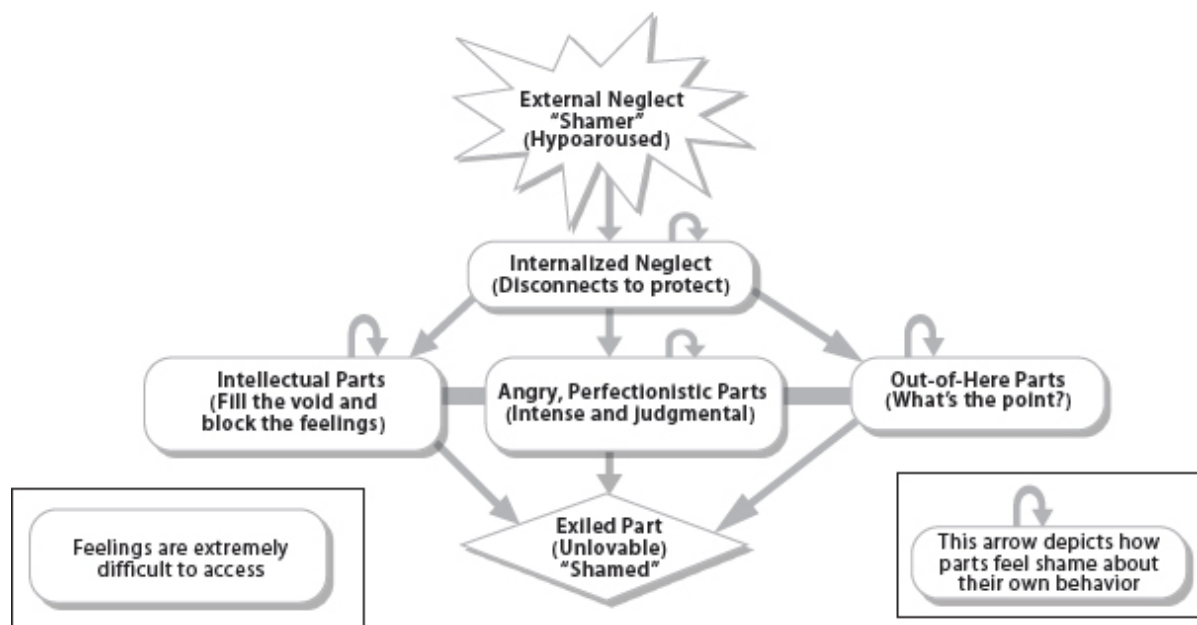
can take a terrible toll on relationships. For instance, if you were assaulted by someone you loved, you are likely to be preoccupied with not getting hurt again and fear opening up to someone new. You may try to hurt them before they have a chance to hurt you (van der Kolk, 2014).

This shame cycle is typically caused by a disconnected, hypoaroused, or withdrawn part of the primary caregiver. It, too, produces a typical range of protective responses within the child in an attempt to protect itself from and avoid the pain of shame and isolation held within the exile. Commonly, an *internalized numb or disconnected part* in the child uses disconnection to escape—a coping mechanism it likely learned from the parent. With the resultant lack of connection, there's often space inside that's filled up by an *intellectual part* trying to understand why a parent doesn't love or care about them. These intellectual parts are often steadfast and have a hard time stepping back and giving room. Remember, “too much space” is problematic for these parts, and giving more space only perpetuates the loneliness.

There are also *perfectionistic parts* that commonly show up and try to solve the old problem of being unwanted by being perfect in the present. Unfortunately, they use outdated or immature behaviors, are often intense, and can come across as judgmental, angry, and frightening when they take over. Again, these parts, by the nature of their behavior, will produce more internalized shame.

There's also the *out-of-here part* to reckon with, the one that thinks: “We've never been loved and never will be. What's the point? It's hopeless. I'm out of here.” In this shame cycle that emerges from abandonment and neglect, feelings are often extremely difficult to access. The client's parts are driven by the absence of unmet needs for love, care, and connection. What they know is nothing! Therefore, accessing emotion is generally foreign to these clients. As a result, it's also difficult for them to take in love when it does show up.

Below is a diagram demonstrating the cycle of shame that originates from neglect.



Shame is not only confined to these two scenarios—one originating from critical verbal abuse and the other from neglect—but it’s often at the core of most complex trauma wounding. The resulting experience of isolation, feeling unworthy and unlovable, and not being seen is physiologically profound and at the core of most shame experiences. We’ve all felt shame at various times in our lives. But our resourceful protective parts build up elaborate and sophisticated systems to help us avoid life-threatening experiences of shame.

Bullying has been prevalent in our culture for so long, and school systems have thankfully started to acknowledge the damaging effects it can have on children. The exiled parts that hold shame are prime targets for being bullied by others. Likewise, the protective parts that develop as a result of being bullied are prime candidates for becoming bullies themselves or for alternatively stopping the pain at all costs through suicide. Unfortunately, many people have been on both sides of this fence, and it’s important to be mindful that at the core of shame is a feeling of being alone and unlovable—yet another example of trauma blocking love.

Ponder, for a moment, the case of Susan, a bright and emotionally intelligent graduate student who came to see me because she felt it was time to work on some childhood issues before starting a family. She was in a long-term relationship that seemed surprisingly stable considering what she had experienced in her youth. Susan had spent time in a religious community that had dismantled about 10 years prior, after several of its members disclosed sexual abuse at the hands of one of the leaders.

Fortunately, Susan had not been abused, but she described the environment as “having an aura of self-righteousness that attracted wounded healers. Everyone was told that we were there because we were ‘chosen,’ and if we connected to the divine daily through prayer, ate healthy, never partook in alcohol or drugs, and fully committed to the leader, we would be enlightened in short order. So I

worked diligently to achieve this goal. When I messed up—let’s say I ate ice cream, snuck a hamburger, or skipped a morning meeting—I felt horrible and frequently would cut myself to get some relief from the pain and to punish myself for what I had done. I knew I wasn’t living up to the ideal of the leader and, once again, had failed. I imagined that I was a huge disappointment to him. Being there reminded me of my family of origin. I could never live up to their standards either and constantly felt like a failure growing up as a child. Here, I was doomed to be a total disappointment all over again.”

As Susan and I explored the origins of her inner critic, we learned that, much to her surprise, two different parts of her produced shame. One part originated from her mother’s harsh and punitive words—this was the part of Susan that believed in embracing physical harm through cutting. The other part came from her father; it was absolute in its thinking, either being “totally right” or “knowing absolutely nothing at all.” Susan added that she felt “like I’m constantly on a roller-coaster ride or I have bipolar disorder or something.” We learned that this part of her was young, around 5 or 6 years old, and developed as a result of her father’s absence. He often traveled for work, and this part of her felt alone and believed that it had to learn things by itself.

When I said that it must have been hard for that little girl to be criticized by her mother and abandoned by her father, Susan instantly agreed. I also shared with her that kids often feel responsible for the way they were treated and asked her to explore if any parts felt at fault. That’s when she burst into tears. She was able to connect compassionately for the first time with both of her critical parts, and she was also capable of appreciating the little girl who felt shamed by them, worthless, and utterly alone. “I see what a huge influence those parts of me have had over my life, both as a little girl and also as an adult in my church community,” she remarked. “I’m so glad we’re getting to the bottom of this because I don’t want to pass any of this down to my children.”

Susan had a parent who was more activated and critical, and another parent who was absent and unavailable—each producing different shaming cycles within her system. Unfortunately, this is a more common occurrence than you think, and these shame systems are often deeply embedded and difficult for clients to unblend from. I believe that clients who are labeled as having borderline personality disorder or who struggle with disorganized attachment often have parents with parts that are harsh and verbally abusive and/or parts that are blunted, neglectful, or abandoning. Both extremes produce shamed exiles in those they interact with, and it’s much more toxic when they coexist together.

As the therapist, it is our job to compassionately hold the space for gentle exploration and to help each part unload the shame it carries toward itself and other parts within the system. If we directly



point out these cycles to clients, we become part of the problem instead of the solution. And we inject even more shame into an already shame-ridden system.

## THE NEUROSCIENCE OF SHAME

An article summarizing the literature on the neurobiological underpinnings of shame found that shame is likely associated with activity in the dorsolateral PFC, posterior cingulate cortex, and sensorimotor cortex (Bastin et al., 2016). Louis Cozolino also describes how the freeze response represents a rapid transition from sympathetic activation to parasympathetic inhibition and that, with shame, children are snapped from a mode of curiosity, exploration, and excitement (mild sympathetic arousal) to one of shutdown and withdrawal (dorsal parasympathetic blunting). He also explains that these “mini traumas” stimulate the same regions of the brain that are activated during pain (Cozolino, 2014). In other words, shame is associated with parasympathetic withdrawal, and we feel it in our body the same way we experience something physically painful.

To demonstrate this point, reflect on John, another client of mine. John was a very successful, high-powered lawyer who came to see me because he wasn’t having enough sex with his wife, wanted to lose some weight, and yearned to get in better shape. I was impressed that he even sought my help because, in my experience, these types of men generally don’t want to talk about their feelings. But there was something different about John, and as he began to tell me his history several red flags popped up that indicated there was more to his story.

On the surface, John was bright and accomplished. He was at the top of his law school class, ran a successful practice, and had a beautiful wife and three happy and healthy children. He was highly intellectual, but occasionally during our sessions he would utter revealing statements like “I hate myself,” “When I fasted for a month in college...,” “I got beat up in school,” and “When my mother was hospitalized for months at a time...” When I heard one of these curious truths emerge, I would pause and encourage John to elaborate and examine that statement. However, John would resist, preferring to only offer facts and stay disconnected from any emotion. John once told me his wife thought he was incapable of feeling, which made me think that John was detached from his feelings on one level, but on another level, there were glimpses of something darker and deeper lurking under the surface.

Over time, John’s intellectual part began to trust him and me more. We were able to learn about his perfectionistic part, which was active toward himself as well as his children. We learned about his exercising and eating parts and their drive to help him achieve his goal weight. We discovered his defensive parts that would often hear feedback as criticism and do their best to make John be seen in a

positive light, no matter what. And we identified his suicidal part that would never let him experience the depression he felt in grade school after he was bullied, choosing instead to insist: “If we get depressed again, I’m out of here. I’ll kill myself for sure.”

Slowly, we began to recognize the hurt and pain these parts were protecting. John was constantly criticized by his father and could never live up to his ideal. These parts yearned for unconditional love. We uncovered the early attachment wounds he suffered when his mother was frequently hospitalized for depression. Family members told him that his mother was hospitalized for more than three months after he was born, and his father frequently left him alone in the crib crying to “teach him how to fall asleep on his own.” To this day, John still goes to sleep holding his “special pillow” like a stuffed animal.

I will never forget the look on John’s face when I asked if he was aware of any parts that may be carrying shame. At that moment, it was as if all his pain had been named for the first time. As we slowly continued to heal the wounds he carried, John’s life began to transform in wonderful and surprising ways. His exercising part took up yoga, while his intellectual part began reading about astronomy—a subject John was never able to pursue due to his need to achieve. He is more honest and emotional with his wife and children, to whom he can now talk about his needs and say no without fear of betrayal or abandonment.

The lesson to be learned here? *Shame is at the root of most relational wounds born from criticism, verbal abuse, abandonment, neglect, and physical and sexual abuse.*

On the next page is a blank template you and your clients can use to keep track of their unique shame cycles.

## HOW SCIENCE VALUES PARTS

In addition to the research on empathy and compassion, Tania Singer and her lab have conducted important research using the IFS concept of parts and have shown that people who are able to identify and connect with their parts are better able to know another person’s perspective and mental state, also known as theory of mind (Böckler et al., 2017). Put another way, the better we understand ourselves, the greater we are able to accurately comprehend the intentions and beliefs of others.

Her endeavor was called the ReSource Project, which examined 332 participants who completed three distinct mental training modules. One, the “presence module,” focused on cultivating present-moment attention and interoceptive awareness (body awareness). The second, the “affect module,” focused on socioaffective qualities of care, compassion, gratitude, and prosocial motivation. The third, the “perspective module,” modeled after IFS, focused on sociocognitive abilities, such as

metacognition and perspective taking, on the Self and others (Böckler et al., 2017). The latter module produced the most robust findings of the three groups, especially when people were able to identify negatively balanced parts of themselves.

Dick Schwartz says that accepting previously unacceptable parts of yourself and others plays a central role in therapeutic success (Schwartz, 2013), which is one of the central tenets of the IFS model. This corroborates what Singer and her group found. Singer states that “an increasingly complex and interconnected world where taking the view of others, especially those from different cultures or different religious backgrounds, becomes ever more difficult and ever more necessary” (Böckler et al., 2017, p. 207).

I believe that IFS is perfectly positioned to be that vehicle of change in the world, and Singer’s research supports this. To be more tolerant and accepting of ourselves and others, even those who are different from us, is what the world desperately needs right now.

